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CHECKLIST · FREE GUIDE

PV Essentials Checklist

A quick reference covering the core pharmacovigilance obligations, ICSR reporting timelines, and compliance checkpoints that every MAH should have in place.

5 SECTIONS 32 CHECKPOINTS 3 PAGES

Pharmacovigilance · Life Sciences · Drug Safety

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01 PV SYSTEM FOUNDATIONS

- ☐ Pharmacovigilance System Master File (PSMF) is current, complete, and accessible to the QPPV.
- ☐ Location of the PSMF is correctly reflected in the Article 57 database.
- ☐ A documented Quality Management System covers all critical PV processes.
- ☐ Standard Operating Procedures exist for every critical activity, are version controlled, and reviewed periodically.
- ☐ Record retention complies with applicable timeframes (EU: marketing authorisation duration plus ten years).
- ☐ Business continuity plan exists and has been tested.

02 THE QPPV

- ☐ A Qualified Person for Pharmacovigilance has been appointed, resides in the EU, and is appropriately qualified.
- ☐ Contact details of the QPPV have been notified to the EMA and member state competent authorities.
- ☐ The QPPV is permanently and continuously available, including out of hours.
- ☐ A deputy QPPV has been designated and the handover process is documented.
- ☐ If the QPPV is not medically qualified, a medically qualified person is accessible.
- ☐ The QPPV has documented oversight of the global PV system, compliance metrics, and audit outputs.

03 ICSR COLLECTION AND REPORTING

- ☐ All four validity criteria are checked for each ICSR (identifiable patient, reporter, suspect drug, suspected reaction).
- ☐ Day zero is consistently calculated across all intake channels.
- ☐ Medical review is documented for every case, not only serious ones.
- ☐ Expectedness assessment uses the current version of the reference safety information.
- ☐ Targeted follow up is performed for cases meeting RMP criteria.
- ☐ Reporting compliance is monitored and late reports trigger documented root cause analysis.

Reporting clocks at a glance:

ICSR TYPE	REPORTING CLOCK
Serious + unexpected (EEA)	15 calendar days
Serious + expected (EEA)	15 calendar days
Non serious (EEA)	90 calendar days
FDA 7-day alert (death/life threatening)	7 calendar days

ICSR TYPE	REPORTING CLOCK
FDA 15-day expedited	15 calendar days
FDA non expedited (PADER)	Periodic

Local timelines may differ. Always verify against the current national legislation for each market.

04 AGGREGATE REPORTS AND RISK MANAGEMENT

- ☐ PSUR / PBRER production schedule is current and aligned with EU reference dates.
- ☐ DSURs are produced annually for products under clinical development.
- ☐ Each applicable product has a Risk Management Plan reflecting current scientific understanding.
- ☐ Additional risk minimisation measures have been implemented and approved before product availability.
- ☐ Effectiveness of risk minimisation measures is measured with leading indicators, not only output metrics.
- ☐ Signal management activities are scheduled per product with intervals appropriate to lifecycle and exposure.
- ☐ Validated signals are tracked through to closure with documented rationale.

05 THIRD PARTIES AND INSPECTION READINESS

- ☐ Safety Data Exchange Agreements (SDEAs) are in place with every PV business partner and service provider.
- ☐ SDEAs define ICSR exchange timelines, responsibilities, and reconciliation frequency.
- ☐ Regular reconciliation is performed and discrepancies are documented and resolved.
- ☐ Audits of contracted partners are conducted on a risk based schedule.
- ☐ A risk based PV audit programme covers the critical processes across a defined cycle.
- ☐ Inspection management procedures are documented and personnel have received refresher training.
- ☐ Self identified findings are closed with effective CAPAs before the next inspection cycle.

HOW TO USE THIS CHECKLIST

This is a high level reference, not a substitute for the GVP modules or applicable national legislation. Use it to identify gaps quickly, then return to the source documents for the detail.

If any item raises a question for your organisation, that is a useful audit signal worth following up.

PRINCIPAL REFERENCES

EMA Good Pharmacovigilance Practices, all current modules
Directive 2010/84/EU and Regulation (EU) No 1235/2010

Commission Implementing Regulation (EU) No 520/2012
ICH E2C(R2), E2D, E2E, E2F
Human Medicines Regulations 2012 (UK), as amended

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